



CONSENT FOR INTERVENTION OR TREATMENT: LIPOSUCTION [0301]

Dr. _____
M.D., Specialist in Plastic and Reconstructive Surgery

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Spanish General Health Act 14/1986 of April 25 (BOE 102), section 10, part 6
Act 41/2002 of November 14, Regulating Patient's Autonomy, and Rights and Duties Concerning
Information and Clinical Documents, chapter IV, sections 8, 9 and 10
Organic Law 15/1999 of December 13, on the Protection of Personal Data

Name: _____

SIGNATURE _____

INSTRUCTIONS

This is a consent document for surgery written by Dr. _____, M.D., Specialist in Plastic, Reconstructive and Aesthetic Surgery, and based on SECPRE (Spanish Society of Plastic, Reconstructive and Aesthetic Surgery) policies, to inform you about LIPOSUCTION (SUCTION-ASSISTED LIPECTOMY SURGERY, ULTRASOUND-ASSISTED LIPECTOMY SURGERY, LASER-ASSISTED LIPECTOMY SURGERY), its risks and alternative treatments.

The processing of both the Spanish General Health Act of 29-04-1986 (BOE 102), section 10, part 6, and of Act 41/2002 of November 14, chapter IV, sections 8, 9 and 10, prior to any assistential procedure is a legal mandatory requirement. The requirements for the Organic Law 15/1999 are collected as well.

Said informed consent will also serve as a CONSIDERATION CONTRACT.

It is important that you read all this information carefully. SIGN every page to indicate that you have read it and SIGN the consent for the surgery recommended by your plastic surgeon, and with which you agree.

OVERVIEW

Liposuction is a surgical technique for the removal of unwanted fat deposits from specific areas of the body, including the face and neck, arms, trunk, abdomen, buttocks, hips and thighs, and the knees, calves and ankles. This is not a substitute for weight reduction, but a method to remove localized deposits of fatty tissue that do not respond to diet or exercise. Liposuction may be performed as a primary procedure for body contouring, or it may be combined with other surgical techniques, such as face lift, abdominoplasty or thigh lift procedures to tighten loose skin and supporting structures.

The best candidates for liposuction are individuals of relatively normal weight who have excess fat in particular body areas. Having firm, elastic skin will result in a better final contour after liposuction. Skin that has lost its tone due to stretch marks, weight loss or natural aging will not reshape itself to the new contours and may require additional surgical techniques to remove and tighten excess skin. Body-contour irregularities due to structures other than fat cannot be improved by this technique.

Liposuction alone will not improve areas of dimpled skin known as "cellulite".

Suction-assisted lipectomy is a surgery performed with a hollow metal surgical instrument known as a *cannula* that is inserted through small skin incision(s), and passed back and forth through the area of fatty deposit. The cannula is attached to a vacuum source, which provides the suction needed to remove the fatty tissue.

In some situations, a special cannula may be used that emits ultrasonic energy to break down fatty deposits. This technique is known as **ultrasonic-assisted lipectomy**. Depending on your needs, your doctor may recommend suction-assisted lipectomy alone, or in combination with ultrasound-assisted lipectomy.

In some situations, a special cannula may be used that emits LASER energy to break down fatty deposits. This technique is known as **LASER-assisted lipectomy**. Depending on your needs, your doctor may recommend suction-assisted lipectomy alone, or in combination with LASER-assisted lipectomy.

There are several techniques used by plastic surgeons for liposuction and care following surgery. Liposuction may be performed with local or general anesthesia. The **tumescent liposuction technique** involves the infiltration of fluid containing diluted local anesthetic and epinephrine into areas of fatty deposits. This technique can reduce discomfort at the time of surgery, blood loss and post-operative bruising.

Support garments (girdles) and dressings are worn to control swelling and stimulate healing. If a liposuction involving the removal of big volumes is expected, your doctor may recommend you to draw a unit of your own blood beforehand, which could be used in case of needing a blood transfusion after the surgery (autologous transfusion).

ALTERNATIVE TREATMENTS

Alternative forms of treatment consist of not treating the areas of fatty deposits. Diet and exercise programs may be of benefit in the overall reduction of excess body fat. In some patients, direct removal of excess skin and fatty tissue may be necessary in addition to liposuction. There are risks and potential complications associated with the alternative forms of treatment that involve surgery.

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RISKS of LIPOSUCTION (Suction lipectomy, Ultrasonic lipectomy, LASER lipectomy)

Every surgical procedure involves a certain amount of risk, and it is important that you understand the risks and possible complications associated with said procedures. In addition, all procedures have limitations. An individual's choice to undergo a surgical procedure is based on the comparison of the risk to the potential benefit. Although most patients do not experience these complications, you should discuss each of them with your plastic surgeon to make sure you understand every possible consequence of liposuction.

Patient selection: Individuals with poor skin tone, medical problems, obesity, or unrealistic expectations may not be candidates for liposuction.

Bleeding: It is possible, though unusual, to have a bleeding episode during or after surgery. Should post-operative bleeding occur, you may require emergency treatment to drain accumulated blood or a blood transfusion. You may require blood transfusions during surgery. Hematomas can occur at any time after surgery and may contribute to infections or other problems. Heparins used to prevent blood clots in veins may cause hemorrhage and a low blood platelet count. Do not take any aspirin or anti-inflammatory medications for ten days before and after the surgery, as these may increase the risk of bleeding. Nonprescription "herbs" and dietary supplements can increase the risk of surgical bleeding. If blood transfusions are needed to treat blood loss, there is a risk of blood-transmitted infections, such as hepatitis and HIV (the virus that causes AIDS). Heparins used to prevent blood clots in veins may cause hemorrhage and a low blood platelet count.

Infection: Infections are rare after the surgery. Should an infection occur, additional treatment, including antibiotics, hospitalization or additional surgery may be necessary. After liposuction surgery, life-threatening infections, including toxic shock syndrome, have been observed in very rare cases.

Scarring: All surgeries leave a scar, some more visible than others. Although the wound is expected to heal well after a surgical procedure, abnormal scars can occur in the skin and deeper tissues. Scars can be unattractive and have a different color than the surrounding skin. The scar appearance can also vary within the scar itself, it can present variations in outline, and it may "frown" due to excess skin. Scars can be asymmetrical (appear different on the right and left sides of the body). There is the possibility of visible marks in the skin caused by sutures. In some cases, scars may require a surgical review or treatment.

Changes in skin sensitivity: It is common to experience a decrease in (or loss of) skin sensitivity in the areas where a surgical procedure was performed. This usually resolves with time. The decrease in (or complete loss of) skin sensitivity is rare, and may not totally resolve.

Skin discoloration/swelling: Bruising and swelling normally occur following liposuction. The skin in or near the surgical site may appear either lighter or darker than the surrounding skin. In rare situations, swelling and skin discoloration may persist for long periods of time. Permanent skin discoloration is rare.

Skin-contour irregularities: Contour irregularities and depressions may occur after liposuction. Visible and palpable wrinkles can also occur in the skin. There is always the possibility of residual irregularities in the skin, on both sides of the incision, or "dog ears", as well as folds of skin when there is too much remaining skin. This can be corrected in time, or through surgical procedure.

Asymmetry: It may not be possible to achieve symmetrical body appearance from liposuction surgery. Factors such as skin tone, fat deposits, bony prominence, and muscle tone may contribute to normal asymmetry in body features. Additional surgery to try to improve asymmetry may be necessary.

Seroma: Fluid accumulations in areas where liposuction has been performed are rare. Additional treatments or surgery to drain accumulations of fluid may be necessary.

Surgical anesthesia: Both general and local anesthesia involves risks. There is the possibility of complications, injury, and even death from all forms of surgical anesthesia or sedation.

Pain: You will experience pain after the surgery. This pain will vary in intensity and duration, and, after liposuction surgery, it may persist. Chronic pain may occur very rarely from nerves becoming trapped in scar tissue.

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Skin sensitivity: After surgery, you may experience itching, pain or exaggerated responses to hot or cold temperatures. This usually resolves during healing, but in rare situations it may be chronic.

Damage to deep structures: In any surgical procedure, there is the possibility of lesions to deep structures, like nerves, blood vessels, muscles and lungs (pneumothorax). The possibility of this happening varies depending on the type of procedure performed. Damage to deep structures may be temporary or permanent.

Slow healing: There may be a rupture or slow healing of the wound. Some areas may not heal normally, and they can take a long time to heal. Some areas of the skin may die. This may require frequent changes of the bandages, or additional surgery to remove unhealed tissue. **Smokers are at a greater risk of skin loss and wound healing complications.**

Allergic reactions: In rare cases, allergic reactions to dressings or bandages, suture material and adhesives, blood products, topical preparations, or injectable agents have been reported. Serious systemic reactions, including shock (anaphylaxis), may occur due to drugs used during surgery and prescription medicine. Allergic reactions may require additional treatment.

Fat necrosis: Deep fatty tissue of the skin may die. This may produce areas of firmness below the skin. Additional surgery may be necessary to remove the areas of fat necrosis. The possibility exists of skin-contour irregularities that may be caused by fat necrosis.

Pubic distortion: It is possible, though unusual, for women to develop distortion of the labia and pubic area. If this happens, additional treatment, including surgery, may be necessary.

Belly Button: Abnormal positioning, scarring, unacceptable appearance or loss of belly button may occur.

Persistent inflammation (lymphedema): After liposuction, persistent inflammation in the legs may occur.

Surgical shock: In rare circumstances, liposuction can cause severe trauma, particularly when multiple or extensive areas are suctioned at one time. Although serious complications are rare, infections or excessive fluid and/or blood loss can lead to severe illness and even death. Should surgical shock occur after liposuction, hospitalization and additional treatment will be necessary. Individuals undergoing a liposuction procedure where a big amount of fat is removed are at a greater risk of complications. For patients considering a big-volume liposuction, over 5,000 cc, a post-operative control and care, including an overnight stay at the hospital, can be recommended.

Deep Venous Thrombosis, Cardiac and Pulmonary Complications: Surgery, especially longer procedures, may be associated with the formation of, or increase in, blood clots in the venous system. Fat embolism syndrome occurs when fat droplets get caught in the lungs. This is a very unusual and possibly fatal complication of suction-assisted lipectomy. Pulmonary complications may occur secondarily to blood clots (pulmonary embolism), fat deposits (fatty embolism) or partial collapse of the lungs after general anesthesia. In some circumstances, pulmonary and fatty embolisms can be life-threatening or fatal. Inactivity and other conditions may increase the incidence of blood clots travelling to the lungs, causing a major blood clot that may result in death. It is important to discuss with your physician any past history of blood clots, swollen legs, or the use of estrogens or contraceptive pills that may contribute to this condition. Cardiac complications are a risk with any surgery and anesthesia, even in patients without symptoms. If you experience shortness of breath, chest pain or unusual heartbeats, seek medical attention immediately. Should any of these complications occur, you may require hospitalization and additional treatment.

Tumescent Liposuction: There is the possibility that large volumes of fluid containing diluted local anesthetics and epinephrine that is injected into fatty deposits during surgery may contribute to fluid overload or systemic reaction to these medications. Additional treatment, including hospitalization, may be necessary.

Ultrasound-Assisted Lipectomy: Risks associated with the use of this technique include the risks mentioned above and the following specific risks:

Burns: Ultrasonic energy may produce burns and tissue damage either at the location where the cannula is inserted into the skin or in other areas if the cannula touches the undersurface of the skin for prolonged periods of time. In case a burn occurs, additional treatment and surgery may be necessary.

Cannula Fragmentation: Ultrasonic energy produced within the cannula may cause disintegration (fragmentation) of the surgical instrument. The occurrence and effect of this are unpredictable. If this happens, additional treatment, including surgery, may be necessary.

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Unknown Risks: The long term effect on tissue and organs of the exposure to short-duration, high-intensity ultrasonic energy is unknown. There is the possibility that additional risk factors of ultrasound-assisted liposuction may be discovered.

LASER-Assisted Lipectomy: Risks associated with the use of this technique include the risks mentioned above and the following specific risks:

Burns: LASER energy may produce burns and tissue damage either at the location where the cannula is inserted into the skin or in other areas if the cannula touches the undersurface of the skin for prolonged periods of time. In case a burn occurs, additional treatment and surgery may be necessary.

Cannula Fragmentation: Ultrasonic energy produced within the cannula may cause disintegration (fragmentation) of the surgical instrument. The occurrence and effect of this are unpredictable. If this happens, additional treatment, including surgery, may be necessary.

Unknown Risks: The long term effect on tissue and organs of the exposure to LASER energy is unknown. There is the possibility that additional risk factors of LASER-assisted liposuction may be discovered.

Unsatisfactory result: Although good results are expected, there is no guarantee, expressed or implied, about the results that may be obtained. You may be disappointed with the results of the liposuction surgery. These may include risks such as asymmetry, unsatisfactory or highly-visible surgical scar location, unacceptable visible deformities, frowning and unevenness of the skin near suture lines or at the ends of the incisions (dog ears), poor healing, wound disruption, and loss of sensitivity. It may not be possible to correct or improve the effects of surgical scars. Additional surgery to try to improve the results may be necessary.

Smoking, Second-Hand Smoke Exposure, Nicotine Products (Patch, Gum, Nasal Spray):

Patients who are currently smoking, use tobacco products, or nicotine products (patch, gum or nasal spray) are at a greater risk for significant surgical complications of skin dying, slow healing and additional scarring. Individuals exposed to second-hand smoke are also at potential risk for similar complications attributable to nicotine exposure. Additionally, smokers may have a significant negative effect on anesthesia and recovery from anesthesia, with coughing and possibly increased bleeding. Individuals who are not exposed to tobacco smoke or nicotine-containing products are at a significantly lower risk for this type of complication. Please indicate your current status regarding these items below:

_____ I do not smoke nor use nicotine products. I understand the potential risk that second-hand smoke exposure causes surgical complications.

_____ I smoke or use tobacco/nicotine products. I understand the risk of surgical complications due to smoking or the use of nicotine products.

It is important to refrain from smoking at least 6 weeks prior to surgery and until the physician tells you it is safe to resume it, if you wish.

ADDITIONAL NOTICES FOR THE PATIENT UNDERGOING LIPOSUCTION / SUCTION LIPECTOMY / ULTRASONIC LIPECTOMY / LASER-ASSISTED LIPECTOMY

Metabolic state of patients who have lost a lot of weight: Your personal metabolic state of blood chemistry and protein levels may be abnormal following massive weight loss and surgical procedures causing the patient to lose weight. Individuals with abnormalities may be at risk of serious medical and surgical complications, including slow wound healing, infection or, in rare cases, even death.

Long-Term Results: Subsequent alterations in your body's appearance may occur as the result of aging, sun exposure, weight loss, weight gain, pregnancy, menopause or other circumstances not related to your surgery.

Female Patient Information: It is important to inform your plastic surgeon if you use birth control pills, replacement therapy with estrogens, or if you believe you may be pregnant. Many medications, including antibiotics, may neutralize the preventive effect of birth control pills, allowing for conception and pregnancy.

Name:

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Intimate Relations After Surgery: Surgery involves the coagulation of blood vessels, and increasing activity of any kind may open these vessels and cause a hemorrhage or hematomas. Any activity that increases your pulse or heart rate may cause additional bruising, swelling, and the need to undergo another surgery to control the bleeding. It is wise to refrain from sexual activity until your physician states it is safe.

Body piercing procedures: Individuals who currently wear body piercing jewelry, or that are planning to undergo a body piercing procedure, are advised that an infection could develop at any time after the surgical procedure. Additional treatment, including antibiotics, hospitalization or surgery may be necessary.

Mental health disorders and elective surgery: It is important that all patients seeking to undergo elective surgery have realistic expectations that focus on improvement rather than perfection. Complications or less-than-satisfactory results are sometimes unavoidable, may require additional surgery and are often stressful. Prior to surgery, openly discuss with your surgeon any history that you may have of significant emotional depression or mental health disorders. Although many individuals may benefit psychologically from the results of elective surgery, effects on mental health cannot be accurately predicted.

Medications: There are many adverse reactions that occur as a result of taking over the counter, herbal, and/or prescription medications. Be sure to check with your physician about any drug interactions that may exist with medications that you are already taking. If you have an adverse reaction, discontinue the drugs immediately and call your plastic surgeon for further instructions. If the reaction is severe, go immediately to the nearest emergency room. When taking prescribed pain medications after surgery, keep in mind that they can affect your thought process and coordination. Do not drive, do not operate complex equipment, do not make any important decisions and do not drink any alcohol while taking these medications. Be sure to take your prescribed medication only as directed.

ADDITIONAL SURGERY NECESSARY

There are many variable conditions that may have an impact on the long-term result from liposuction. It is unknown how the treated tissues may respond, or how the wound will heal after the surgery. Secondary surgery to optimize the results may be necessary. In case a complication occurs, additional surgery or other treatments may be necessary. Even though risks and complications are rare, the risks mentioned are particularly associated with liposuction or lipectomy. Other complications and risks can occur, but they are even more uncommon. The practice of medicine and surgery is not an exact science. Although good results are expected, there is no guarantee, expressed or implied, about the results that may be obtained. In some cases, it may not be possible to get optimum results with one single surgical procedure.

PATIENT COMPLIANCE

Follow your physician's instructions carefully; this is essential for the success of your outcome.

It is important that surgical incisions are not subjected to excessive force, swelling, abrasion, or motion during the healing period. Personal and work activities must be restricted. Protective dressings and drains should not be removed unless instructed by your plastic surgeon. Successful post-operative function depends on both the surgery and subsequent care. Any physical activity that increases your pulse or heart rate may cause additional bruising, swelling, fluid accumulation and the need to undergo another surgery. It is wise to refrain from intimate physical activities after surgery until your physician states it is safe. It is important that you participate in follow-up care, that you return for aftercare, and that you promote your recovery after surgery until your final medical discharge (approximately 1 year).

Name:

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FINANCIAL RESPONSIBILITIES

The cost of the surgery includes several charges for the services provided. The total includes your doctor's fees, the cost of surgical materials, the anesthesia, laboratory tests, and outpatient hospital charges, depending on where the surgery was performed. Based on whether the cost of surgery is covered by your medical insurance, you will be responsible for the necessary co-payments, deductibles and charges that are not covered. The charges for this procedure don't include possible future costs for additional procedures of your choice, or that you require to review, optimize or complete your result. Additional costs may occur should complications develop from the surgery. Secondary surgery charges or hospital day surgery charges related to revision surgery will also be your responsibility. By signing the consent for this surgery/procedure, you acknowledge you have been informed about its risks and consequences, and take responsibility for the medical decisions made, together with the economic costs of any future treatments.

DISCLAIMER

Informed consent documents for surgery are used to inform about the recommended surgical treatment of a disease or condition along with the statement of risks and alternative forms of treatment(s). The informed consent process for surgery attempts to define the principles of a statement of risks that usually should meet the needs of most patients undergoing a blepharoplasty in most circumstances. However, informed consent documents for surgery should not be considered all-inclusive in defining other methods of care or risks encountered. Your plastic surgeon may provide you with additional or different information, based on all the facts in your particular case, as well as the state of your medical knowledge. Informed consent documents for surgery are not intended to define or serve as the standard of medical care. Standards of medical care are determined on the basis of all of the facts involved in an individual case, and are subject to change as scientific knowledge and technology advance and as medical practice patterns evolve.

It is important that you read the above information carefully and have all of your questions answered before signing the CONSENT FOR INTERVENTION OR TREATMENT on the next page.

Name:

SIGNATURE

INFORMED CONSENT FOR INTERVENTION or TREATMENT

Name of Patient:

Date:

1. I hereby authorize Dr. _____ and such assistants as may be selected to perform the following surgery or treatment:

ICE9MC-
ICE9MC-
ICE9MC-
ICE9MC-

due to the following diagnoses:

ICE9MC-
ICE9MC-
ICE9MC-
ICE9MC-

That will be carried out on _____ at the CLINIC _____

I have received, read and understood the following information that has been given to me:

INFORMED CONSENT FOR INTERVENTION — LIPOSUCTION
SECPRE LEAFLET
AECEP LEAFLET
LEAFLET about _____ – LIPOSUCTION

2. I RECOGNIZE that, during the intervention, unforeseen conditions may necessitate different procedures than those described above. Therefore, I HEREBY EXPRESSLY AUTHORIZE said procedures, including biopsies, X-rays, blood transfusions, etc. In case of complications during the operation, I AUTHORIZE the physician mentioned above to request the assistance he/she deems necessary from other specialists, in their best professional judgment. I understand that, despite having chosen the right technique and being correctly performed, there may be unwanted effects or unexpected clinical findings, making it necessary to perform other interventions or adopt other therapeutic measures not only to control the procedure's results, but also to ensure my wellbeing.

3. I authorize the administration of such anesthetics considered necessary and advisable. I understand that all forms of anesthesia involve a risk and the possibility of complications, lesions, and sometimes death.

4. I acknowledge that I haven't been given any guarantee as to the results that may be obtained.

5. I CONSENT to be photographed or filmed prior, during and after the treatment itself, as well as to check the progress of the case, this material being a graphic means to record my defect or deformity, property of the surgeon, and could be published in journals and scientific books, or being exhibited for medical purposes, NEVER IN THE DAILY PRESS OR REGULAR MAGAZINES, except with MY EXPRESS PERMISSION.

6. I AGREE to follow the surgeon's instructions to the letter as best as I can before, during and after the procedure mentioned above, and until I am medically discharged. I understand and accept that post-operative visits are ESSENCIAL AND FUNDAMENTAL to achieve a good final result. I agree to follow these to the letter, as well as all post-operative recommendations, until I am medically discharged.

7. I ATTEST I have not left out or altered any data when showing my surgical medical history and records, especially those concerning allergies and diseases or personal risks, including the intake of medications, toxics or any other kind of substances.

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8. I ACCEPT the surgeon postpones or cancels the operation, if he/she deems it necessary.

9. I have clarified any doubts about everything that has been mentioned above, and I fully understand this CONSENT DOCUMENT, standing by each and all of its points or WITH THE EXPRESS EXCEPTION _____

_____ mentioned on page _____, item _____.

10. I UNDERSTAND that the goal of the operation is TO IMPROVE MY APPEARANCE, with the possibility that some imperfection remains and that the result may not be what I expected. I am aware that the Medical field is not an exact science, and that absolute perfection cannot be guaranteed. I acknowledge that I haven't been given any guarantee at all. I also understand, and it has been duly explained to me, that a second and even successive surgical times for small corrections may be necessary in the future to obtain a good final result.

11. THE FOLLOWING HAS BEEN EXPLAINED TO ME IN A WAY THAT I UNDERSTAND:

- a. THE ABOVE TREATMENT OR PROCEDURE TO BE UNDERTAKEN
- b. THAT THERE MAY BE ALTERNATIVE PROCEDURES OR METHODS OF TREATMENT.
- c. THERE ARE RISKS TO THE RECOMMENDED PROCEDURE OR TREATMENT
- d. THE LOCATION OF SCARS.

12. I CONFIRM that the operation mentioned above has been fully explained to me by the surgeon in words I can understand, including risks involved, other alternative solutions to the treatment (if there are any), and also discomfort or, occasionally, pain I may feel in a normal post-operative setting. I have understood the explanations given to me in clear, simple words, and the physician attending me has let me make all the observations and has clarified any and all doubts I had.

13. I EXPRESSLY CONSENT to the handling of my personal data, which, pursuant to Organic Law 15/1999, will be gathered for the purpose of appropriately managing the intervention and pre- and post-operative process, and included in records belonging to Dr. _____. I declare that all the information I have provided is true and correct, and I agree to communicate any changes made to them. Furthermore, to carry through said intervention and treatment, I expressly consent to transfer my data to as many professionals as necessary to perform relevant consultations. DR Privacy Policy. _____ ensures the Patient may exert, in any case, his/her right of access, rectification, cancellation and objection of his/her information by notifying Dr. _____ via regular postal mail to DR. _____ C/ _____, Zip Code _____ - CITY _____ (Prov. _____), or by e-mail: _____@_____.

14. THIS DOCUMENT MUST BE SIGNED AND DELIVERED AT CONSULT AT LEAST 24 HOURS PRIOR TO THE INTERVENTION. OTHERWISE, IT IS PRESUMED THAT THE PATIENT FULLY AGREES WITH WHAT HAS BEEN SET FORTH HERE.

I CONSENT TO THE TREATMENT OR PROCEDURE, AND TO THE ITEMS LISTED ABOVE (1-14), HAVING READ AND UNDERSTOOD IN PLAIN ENGLISH THE INFORMATION DESCRIBED IN THE 9 PAGES OF THIS CONSENT. I HAVE BEEN ASKED IF I WANT A MORE DETAILED EXPLANATION, BUT I AM SATISFIED WITH THE EXPLANATION AND DO NOT WANT MORE INFORMATION.

PATIENT

WITNESS (or authorized person)

Name:
ID number:

SIGNATURE:

Name:
ID number:

SIGNATURE: